G1 CLIMBING + FITNESS · PERSONAL TRAINING

First-Session Assessment

Trainer use. A NASM-informed starting point for programming, not a medical exam or diagnosis.

SAMPLE · DRAFT V0.1
Pending G1 management review

Client: Casey Sample | Date: 09/08/2026 | Session length (min): 60

Trainer: Taylor Trainer (sample)

Scope. Personal trainers observe movement to plan exercise. We don't diagnose, treat or rehabilitate injuries. Describe what you see in plain words (for example "left shoulder sits higher"), and refer out when something is outside our scope.

1 · BEFORE WE START

- [x] Intake reviewed with the client
- [ ] Waiver and policies on file (per G1 process)
- [x] Client knows they can skip or stop anything
- [x] Goals confirmed in the client's own words
- [x] Explained what today is, and isn't
- [x] Hands-on adjustment preference checked

PLACEHOLDER: G1 process for waiver on file, incident reporting and medical-clearance requirements. Pending management approval.

2 · TODAY'S CHECK-IN

Energy today (1-5)	Sleep last night (hrs)	Pain right now (0-10)	Where?
4	6.5	0	none

Anything changed since the intake form? Nothing new

Decision

- (●) Proceed as planned
- ( ) Proceed with modifications
- ( ) Pause and suggest a healthcare check-in

Stop and refer. Pause the session if the client reports chest pain or pressure, fainting or severe dizziness, unusual shortness of breath, sudden sharp or radiating pain, new numbness or tingling, or a joint that feels unstable after an injury. Don't diagnose or guess. Suggest they see a licensed healthcare professional, note what happened, and follow G1's incident process **[placeholder: pending management]**.

3 · RESTING MEASURES (optional; record only what helps the client's goals)

Resting heart rate (bpm)	Blood pressure (if G1 policy allows)	Height	Weight (only if the client wants it)
68			

PLACEHOLDER: G1 policy on blood pressure and body measurements: who may take them, any thresholds, and how results are recorded.

4 · POSTURE SNAPSHOT (optional, standing; describe, don't label)

CHECKPOINT	WHAT I SEE (FRONT, SIDE, BACK)
Feet and ankles	Feet turn out slightly, right more than left
Knees	Level, no obvious difference side to side
Hips and low back	Pelvis looks slightly tipped forward from the side
Shoulders	Both sit slightly forward; right a little higher
Head and neck	Head sits slightly forward of the shoulders

SAMPLE — PROFESSIONAL CLIENT

Client: Casey Sample

Observe, don't diagnose. These screens show how the client moved today, in these shoes, with these instructions. Use them to choose a starting point and to re-check later. Stop any test that causes pain.

A · Overhead squat 5 slow reps, feet shoulder-width, toes forward, arms overhead. Watch from the front, side and back.

Front: feet and knees	☐ Feet turn out ☑ Knees move inward ☐ Knees move outward
Side: hips and low back	☐ Excessive forward lean ☐ Low back arches ☐ Low back rounds
Side: shoulders	☑ Arms fall forward
Back: feet and hips	☐ Feet flatten ☐ Heels rise ☐ Weight shifts to one side
Overall	○ Smooth ◉ Minor compensation ○ Clear compensation ○ Stopped (pain or limit)
Depth	◉ Thighs about parallel ○ Limited
Pain during the squat	◉ None ○ Yes, where: _____

B · Single-leg squat Hands on hips, squat to a comfortable depth, 5 reps each side. Tick what you see.

OBSERVATION	LEFT	RIGHT
Knee moves inward	☐	☑
Knee moves outward	☐	☐
Hip hikes or drops	☐	☐
Trunk leans or low back arches	☐	☐
Loses balance	☐	☑
Pain or discomfort (note where below)	☐	☐

C · Pushing and pulling A push-up or standing band/cable press, then a row or band pull. Watch from the side.

Push	☐ Low back arches ☑ Shoulders rise toward ears ☐ Head moves forward
Pull	☐ Low back arches ☑ Shoulders rise toward ears ☐ Head moves forward

D · Climbing add-ons Optional. Skip any that cause pain. No max-effort finger or hangboard testing in session one.

SIDE-BY-SIDE CHECKS	LEFT	RIGHT
Knee-to-wall distance, heel down (cm)	11	9
Seated trunk rotation limited?	☐	☑

Overhead reach (back to wall)	☐ Reaches up without arching ☑ Ribs flare or low back arches ☐ Can't reach fully
Supported hang (feet on floor, 10 s or less)	☐ Shoulders stay set ☑ Shoulders shrug to ears ☐ Skipped

What stood out, and what the client said they felt (including any pain or discomfort, with where)

No pain in any test. Knees tracked better after one cue ("push knees toward little toes"). Client said the overhead position feels "stuck" and the right ankle feels tighter. Mentioned the right wrist again: no pain today, so we'll monitor it and avoid max-effort finger loading in block 1.

SAMPLE — FICTIONAL CLIENT

Client: Casey Sample

5 · BASELINE SNAPSHOT (optional; pick what fits; quality before quantity; stop on pain)

TEST	VERSION / SETUP	RESULT	NOTES (FORM, EFFORT, RPE)
Plank hold	Forearm plank	40 s	Steady; hips sagged near the end
Push-up (wall, incline, knees, full)	Incline (bench)	10 reps	Shoulders rose on the last 3 reps
Sit-to-stand, 30 seconds	Standard chair	14 reps	Easy, RPE 3
Easy cardio, 5 minutes	Bike, level 3	RPE 4, HR 118	Passed the talk test easily
Climbing movement (optional)	Not done today		Save for session 3

6 · WHAT I NOTICED, AND THE TRAINING FOCUS (hypotheses to test through programming, not diagnoses)

WHAT I NOTICED	STARTING EMPHASIS IDEA	PRIORITY
Knees move inward (overhead squat, right single-leg)	Hip abductor and glute activation; ankle mobility; knee-tracking cues	H
Arms fall forward; shoulders shrug in the hang	Lat and chest length; mid-back and rotator-cuff activation; thoracic mobility	H
Shoulders rise in push and pull	Scapular control: band pull-aparts, cue "long neck"	M
Right ankle feels tighter	Calf and ankle mobility; knee-to-wall drills	M

QUICK REFERENCE: COMMON OBSERVATIONS AND WHERE PROGRAMMING OFTEN STARTS (check against your NASM materials)	
Feet turn out or flatten; heels rise	Calf and ankle mobility; foot and lower-leg activation; stance cues
Knees move inward	Hip abductor and glute activation; foot and ankle mobility; knee-tracking cues
Low back arches; excessive forward lean	Hip flexor and lat length; glute and deep-core activation; rib and pelvis position cues
Low back rounds	Hamstring and hip mobility; glute and back-extensor activation; hip-hinge practice
Arms fall forward	Lat and chest length; mid-back and rotator-cuff activation; thoracic mobility
Shoulders rise or head moves forward (push, pull, hang)	Upper-trap and neck length; scapular control work; deep neck-flexor activation

7 · STARTING PLAN

Starting OPT phase (most new clients start in Phase 1)

(●) Phase 1 Stabilization Endurance ( ) Phase 2 Strength Endurance ( ) Other: _____

Why this phase: New to structured training; build movement quality and control first

Sessions per week: 2

Homework (1-2 simple items): Glute bridge 2 × 12 and wall slides 2 × 10, most days

First progress review: 10/06/2026 (week 4)

Client's own words after today: what felt good, what worried them: Felt good, no pain. Excited about the December trip. A little worried about the right wrist.

Referral or follow-up (a suggestion only; document what you said)

(●) None needed ( ) Suggested a healthcare check-in about:

Next session (date, time): Thu 09/10/2026, 6:00 pm

Referral topic (if any)

OPT phases: 1 Stabilization Endurance · 2 Strength Endurance · 3 Muscular Development · 4 Maximal Strength · 5 Power. This sheet is NASM-informed. It is not affiliated with or endorsed by NASM, and OPT is a trademark of NASM.

SAMPLE — FICTIONAL CLIENT